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Clinical Diagnosis & History:

year old woman with right breast mass s/p core bx -- IFDC poorly differentiated.

Specimens Submitted:

- 1: SP: Sentinel node #1, level 1, rt. axilla (fs)
- 2: SP: Sentinel node #2, level 1, rt. axilla (fs)
- 3: SP: Rt. breast ;

UUID: FE9C56C4-B6D4-4054-A58C-13642370052E
TCGA-AO-A030-01A-PR

Redacted



DIAGNOSIS:

- 1) SENTINEL LYMPH NODE #1, LEVEL I, RIGHT AXILLA; BIOPSY:
 - ONE BENIGN LYMPH NODE (0/1).
 - ADDITIONAL H&E STAINED SECTIONS AND IMMUNOHISTOCHEMICAL STAINS FOR CYTOKERATINS (AE1:AE3 AND CAM 5.2) SHOW NO EVIDENCE OF METASTATIC TUMOR.
- 2) SENTINEL LYMPH NODE #2, LEVEL I, RIGHT AXILLA; BIOPSY:
 - ONE BENIGN LYMPH NODE (0/1).
 - ADDITIONAL H&E STAINED SECTIONS AND IMMUNOHISTOCHEMICAL STAINS FOR CYTOKERATINS (AE1:AE3 AND CAM 5.2) SHOW NO EVIDENCE OF METASTATIC TUMOR.
- 3) BREAST, RIGHT AND LEVEL I LYMPH NODES; MASTECTOMY AND AXILLARY DISSECTION:
 - INVASIVE DUCTAL CARCINOMA, NOS TYPE, HISTOLOGICAL GRADE III/III (SLIGHT OR NO TUBULE FORMATION), NUCLEAR GRADE III/III (MARKED VARIATION IN SIZE AND SHAPE), MEASURING 3.5 CM IN LARGEST DIMENSION GROSSLY.
 - DUCTAL CARCINOMA IN SITU (DCIS) IS ALSO IDENTIFIED, SOLID TYPE, WITH HIGH NUCLEAR GRADE AND EXTENSIVE NECROSIS. THE DCIS CONSTITUTES <= 25% OF THE TOTAL TUMOR MASS.
 - THE INVASIVE CARCINOMA IS LOCATED IN THE UPPER OUTER QUADRANT
 - NO INVOLVEMENT OF THE NIPPLE BY EITHER IN SITU OR INVASIVE CARCINOMA IS IDENTIFIED.
 - CALCIFICATIONS ARE PRESENT IN THE IN SITU AND INVASIVE CARCINOMA AND IN BENIGN BREAST PARENCHYMA.
 - LYMPHOVASCULAR INVASION IS PRESENT.
 - PERINEURAL INVASION IS PRESENT.
 - NO INVOLVEMENT OF THE SURGICAL MARGINS BY EITHER INVASIVE OR IN SITU CARCINOMA IS IDENTIFIED.

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1CD-0-3

carcinoma, infiltrating duct, nos 8500/3
Site: breast, nos 250.9 lw
10/21/11

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Notes		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	lw	10/21/11

- NO SKIN INVOLVEMENT BY CARCINOMA IS IDENTIFIED.
- THE NON-NEOPLASTIC BREAST TISSUE SHOWS STROMAL FIBROSIS AND FLORID DUCTAL HYPERPLASIA.
- THE LYMPH NODE STATUS IS AS FOLLOWS (EXPRESSED AS THE NUMBER OF POSITIVE LYMPH NODES IN RELATION TO THE TOTAL NUMBER OF LYMPH NODES EXAMINED): LEVEL I: 0/15.
- IMMUNOHISTOCHEMICAL STUDIES FOR ESTROGEN AND PROGESTERONE RECEPTORS (ER/PR) ARE POSITIVE.
- IMMUNOHISTOCHEMISTRY FOR HER2/NEU (HERCEPT) IS BEING REPEATED AND WILL BE REPORTED IN AN ADDENDUM.

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Special Studies:

Result	Special Stain	Comment
	CAM 5.2	
	CAM 5.2	
	AE1:AE3	
	AE1:AE3	
	NEG CONT	
	IMM RECUT	
	NEG CONT	
	IMM RECUT	
	CAM 5.2	
	AE1:AE3	
	NEG CONT	
	IMM RECUT	
	ER-C	
	PR-C	
	HER2-C	
	NEG CONT	
	NEG-HER2	
	IMM RECUT	

Gross Description:

- 1) The specimen is received fresh for frozen section, labeled "Sentinel node #1, level 1, rt. axilla". It consists of single lymph node measuring 2.5 x 0.5 x 0.3 cm. Entirely frozen.

Summary of Sections:

FSCA and B - frozen section control A and B

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2) The specimen is received fresh for frozen section, labeled "Sentinel node #2, level 1, rt. axilla". It consists of of single lymph node measuring 2.5 x 0.5 x 0.3 cm. Entirely frozen.

Summary of Sections:

FSCA and B - frozen section control A and B

3) The specimen is received fresh, labeled, "Rt. breast (stitch marks level one lymph nodes)". It consists of a right mastectomy consisting of tan skin, breast tissue and axillary tail designated as level one. The skin measures 27.0 x 16.0 cm. The nipple is everted. No visible scars are present on the skin surface. No skeletal muscle tissue is present at the deep margin, which is inked. The breast tissue is sectioned to reveal an indurated, irregular gray-white tumor, situated in the upper outer quadrant, 4.0 cm from the deep margin and measuring 3.5 x 3.0 x 3.0 cm. The rest of the breast tissue is fibrofatty. The axillary tail is sectioned to reveal multiple lymph nodes ranging from 0.8 up to 1.5 cm in greatest dimension. The largest node is bisected. Tumor and normal tissue taken for TPS. Representative sections are submitted.

Summary of Sections:

N nipple

T tumor

DM deep margin closer to the tumor

UOQ - upper outer quadrant

LOQ - lower outer quadrant

UIQ - upper inner quadrant

LIQ - lower inner quadrant

LNI lymph nodes level one

BLNI bisected lymph nodes level one

Summary of Sections:

Part 1: SP: Sentinel node #1, level 1, rt. axilla (fs)

Block	Sect.	Site	PCs
1		FSCA	1
1		FSCB	1

Part 2: SP: Sentinel node #2, level 1, rt. axilla (fs)

Block	Sect.	Site	PCs
1		FSC	1

Part 3: SP: Rt. breast

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Block	Sect.	Site	PCs
1		BLN1	2
1		DM	3
2		LIQ	2
3		LN1	15
2		LOQ	2
1		N	2
3		T	3
2		UIQ	2
2		UOQ	2

Procedures/Addenda:

Addendum

Date Ordered:

Status: Signed Out

Date Complete:

By:

Date Reported:

Addendum Diagnosis

ADDENDUM

3) BREAST, RIGHT; MASTECTOMY

HER2/NEU (HERCEPTEST): NEGATIVE (STAINING INTENSITY OF 1+).
[CONTROL IS SATISFACTORY.]

Intraoperative Consultation:

Note: The diagnoses given in this section pertain only to the tissue sample examined at the time of the intraoperative consultation.

- 1) FROZEN SECTION DIAGNOSIS: NEGATIVE FOR CARCINOMA.
PERMANENT DIAGNOSIS: SAME
- 2) FROZEN SECTION DIAGNOSIS: NEGATIVE FOR CARCINOMA.
PERMANENT DIAGNOSIS: SAME

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** End of Report **